



Cytopathology & Flow Cytometry Report – Lymph Node FNA

Department: Laboratory Medicine – Cytopathology

Accession: NSHN■CYTO■21■10477

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Ordering Clinician: ENT / Head & Neck Surgery

Specimen Site: Left neck, level II lymph node

Fixation/Handling: Air■dried and alcohol■fixed smears; cell block prepared

****Specimen:**** Fine needle aspirate, left cervical lymph node (zone II), ultrasound-guided. Two direct smears and a cell block were prepared; a separate tube was submitted for flow cytometry.

****Clinical History (provided):**** Persistent cervical lymphadenopathy with ultrasound showing a dominant level II node with cortical thickening. Request: exclude lymphoma or metastatic malignancy.

****Cytology (Microscopy):**** Smears show a polymorphous lymphoid population with small mature lymphocytes, scattered centroblast-like forms, and occasional immunoblasts. Tingible-body macrophages are present, supporting reactive follicular activity. No granulomas are identified. No necrosis is seen. No cohesive epithelial clusters are present to suggest metastatic carcinoma.

****Cell Block:**** Limited cellularity; fragments of lymphoid tissue show reactive-appearing lymphocytes without overt atypia. Architecture assessment is limited by the nature of FNA material.

****Ancillary Stains:**** Papanicolaou stain negative for malignant epithelial cells. No organisms identified on routine review (special stains not requested on this specimen).

****Flow Cytometry:**** Predominantly T-cells with ****CD4:CD8 ratio approximately 3.13:1****. A small B-cell population is present without aberrant marker expression; no clear kappa/lambda light-chain restriction is detected. No immunophenotypic evidence of a clonal B-cell lymphoproliferative disorder is demonstrated.

****Final Interpretation:**** ****Negative for malignancy.**** Findings support a ****reactive lymphoid population****. Correlate with imaging and clinical course. If lymph nodes continue to enlarge, are persistent beyond expected reactive timelines, or if systemic symptoms develop, consider repeat sampling or excisional biopsy for nodal architecture and broader ancillary testing.

****Limitations / Comment:**** Sampling error is possible. FNA may miss focal involvement, and some indolent lymphomas can be difficult to diagnose without excision.



Addendum / Reporting Notes

This document uses standardized institutional templates to emulate real-world medical record formatting for prototype testing. All names, identifiers, and institutional details are fictional. Clinical content is synthesized from the provided longitudinal summary.

If results appear internally inconsistent across documents, this is intentional to mimic typical real-world fragmentation. When discrepancies arise, the recommended approach is to reconcile using original source dates and the most recent verified medication/problem lists.